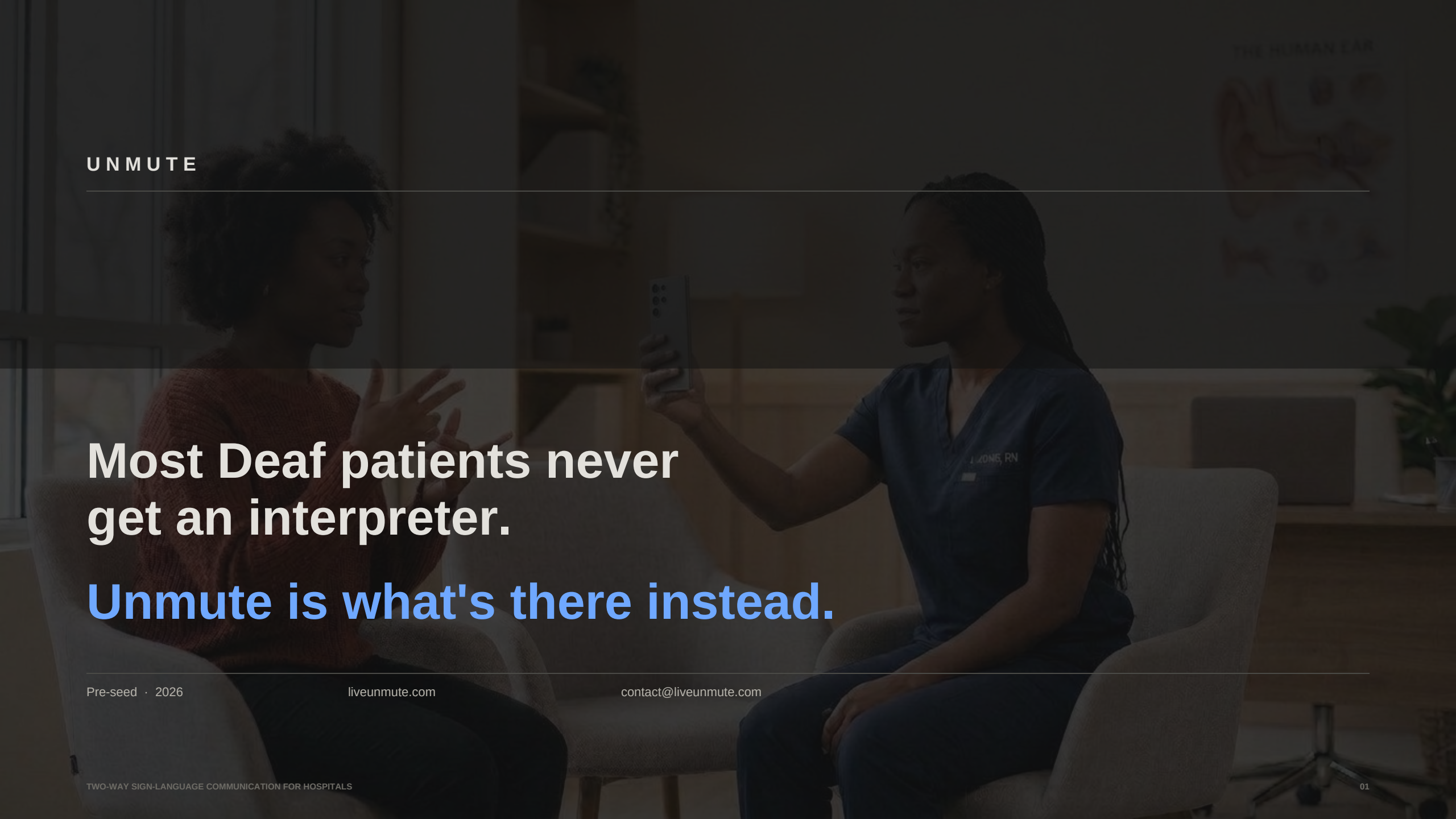


UNMUTE

Most Deaf patients never get an interpreter.

Unmute is what's there instead.

Pre-seed · 2026

liveunmute.com

contact@liveunmute.com

Under one in ten encounters involves a professional interpreter.



How emergency physicians actually communicate with Deaf patients (n = 166). Akeely et al., Cureus, 2022. In the same study, 74.1% of departments had no policy for Deaf patients and 90.4% of staff knew no sign language.

THE ARITHMETIC

Fifty Deaf ASL users for every certified interpreter.

500,000

Deaf adults in the US who use ASL as a primary language

10,000

Certified ASL interpreters in the US

50 : 1

Before accounting for geography, hours, or scheduling

A million deaf and hard-of-hearing adults use some form of signing. We quote the smaller, defensible figure of 500,000 on purpose.

SOURCES: AAFP 2024 · REGISTRY OF INTERPRETERS FOR THE DEAF

In the vacuum, care is improvised — and sometimes it goes wrong.

> *“Interpreter access was uncertain, delayed or absent at key points of emergency care, including assessment and consent.”*
>
> HAZELWOOD ET AL., JOURNAL OF CLINICAL NURSING, 2026

> *A triage delayed awaiting an ASL interpreter. A Deaf patient restrained for a scan, unable to tear skin trying to get free.*
>
> PENNSYLVANIA PATIENT SAFETY AUTHORITY, 2025

What fills the gap in practice: handwritten notes · lip-reading · a relative's phone · gestures

General sign models fail exactly where clinical communication lives.

VOCABULARY

Trained on broadcast and lecture footage. Knows family, school, weather.

Has never seen hypertension, NPO after midnight, or a fingerspelled drug name.

NUMERACY

Fifteen milligrams and fifty milligrams differ by one handshape.

Dosage is the highest-consequence token in the room.

GRAMMAR

ASL has its own syntax and non-manual markers.

Anything that transcribes word-for-word produces signed English, which Deaf users correctly reject.

Two hundred signs and phrases carry most of the routine clinical risk.

PAIN
Location, severity, onset, duration

HISTORY
Allergies, current medication, prior conditions

CONSENT
Affirmation, teach-back, understanding check

DISCHARGE
Dose, frequency, warning signs, follow-up

Bounded vocabulary. High stakes.
Repeated thousands of times a day, in every hospital in the country.

Three flows. One direction. Measured, not promised.

v1 SHIPS

- Sign → text, on the patient's device
- Sign → speech, for the clinician
- Clinician → patient via recorded human signing, fixed clinical phrase set
- Runs on the hardware already in the room

NOT IN v1

- Generative signing avatars
- Text → sign synthesis
- Multilingual
- EHR integration
- Offline mode

A QUALIFIED INTERPRETER IS STILL ENGAGED

- Consent for procedures
- Diagnosis disclosure
- Mental health assessment
- Complex discharge planning
- End-of-life discussions

What exists, and what doesn't.

EXISTS

- Problem definition, grounded in published clinical evidence
- Vendor landscape mapped and priced for both directions
- Clinical vocabulary scope defined
- A founder who has worked this problem for a year

DOES NOT EXIST

- A trained model
- A published clinical accuracy number
- A pilot
- A signed design partner
- Revenue

The only defensible asset here is a corpus nobody can scrape.

01	**DATA**	Consented clinical ASL. Deaf patients and qualified interpreters, recording clinical vocabulary in clinical context, annotated to a protocol written with an ASL linguist, regional variation preserved rather than averaged away.
02	**BENCHMARK**	The published clinical accuracy number becomes what procurement asks every vendor for.
03	**WORKFLOW**	Embedded in triage. Switching cost rises with every shift that runs on it.
04	**INSTITUTION**	Security review completed, BAA signed, procurement navigated. Paperwork is a moat when your competitor is a solo founder.

Subscription, per care team. Priced against the encounter it covers.

WARD

/ mo

Up to 10 practitioners

Nurse manager, clinic manager, ADA coordinator

DEPARTMENT

~$599 / mo

Up to 50 practitioners

BUYER

ENTERPRISE

Custom

50+ practitioners

BUYER

Billed monthly · annual prepay discount · BAA included from the Ward tier up.

These prices are a hypothesis, being tested in ten conversations.

The current alternative is free. That is the thing to answer.

> *“A family member does it for free.”*

ASPECT	CURRENT PRACTICE	UNMUTE
COST	$0	~$199/month, up to 10 practitioners
COMPLIANCE POSTURE	Discouraged by the National Association of the Deaf — no impartiality, no medical vocabulary, no capacity for distress	Purpose-built, documented, auditable
RECORD OF THE ENCOUNTER	None	Communication log
RISK IF IT GOES WRONG	The hospital chose the free option knowingly	A documented attempt at communication

Three design partners, not a sales team.

0 – 6 MONTHS

Design partners

Three departments — one ED, one outpatient clinic, one community health centre. Free for six months. In exchange: workflow access, data collection consent, a named testimonial.

6 – 12 MONTHS

Paid ward deployments

Bottom-up. A nurse manager buys the Ward tier on a card. No procurement committee, no RFP, no six-month cycle.

12 – 24 MONTHS

Enterprise

The first two steps generate the security review, the BAA template and the reference customer that enterprise sales requires.

Channels: Deaf community organisations and interpreter agencies as referrers — not competitors.

Eighteen months. Three milestones. One number we're willing to publish.

M1 · 0–6 MONTHS

Clinical ASL model, 200-term scope

Accuracy measured on held-out clinical data.

Proof: the number is published, with its failure modes.

M2 · 6–12 MONTHS

Three design partners live

Deaf user and clinician testing in real departments.

Proof: measured time-to-communication, not satisfaction scores.

M3 · 12–18 MONTHS

First paid contract and corpus

One Ward-tier customer paying, and an annotated corpus at a defensible size.

Proof: revenue, and data a competitor cannot buy.

> *If we miss M1, you'll know before we do — because we'll have published the number.*

One founder today. The gap is the pitch.

TODAY

Founder — AI research, NLP, product
Built the ASL data pipeline. One year inside this problem.

BEING RECRUITED — BEFORE OR ALONGSIDE THIS ROUND

Deaf co-founder / Head of Product
Decision authority, not an advisory title.

Clinical advisor
Emergency medicine or nursing, currently practising.

ASL linguistics advisor
Owns the corpus annotation protocol.

Paid Deaf design panel
5–8 people, paid, with veto power over releases.

THE ASK

Raising $250,000 to reach a number we can publish.

$250k

Pre-seed · 18 months

Built bottom-up from the six lines below, not chosen as a round figure.

USE OF FUNDS	AMOUNT	SHARE
Corpus collection & annotation — 200 terms, 5+ signers, linguist-annotated	**$85,000**	34%
Founder runway & operations	**$45,000**	18%
Clinical validation — three design partners	**$35,000**	14%
Compliance — HIPAA posture, BAA, security review, counsel	**$30,000**	12%
Deaf design panel & advisory — ASL linguistics, clinical	**$30,000**	12%
Model development & compute	**$25,000**	10%

Staged, not lumped: the first $110,000 buys the corpus and the compute — that is M1, the published accuracy number.

APPENDIX A1

Evidence and sources.

[1] Graham Center, “Understanding the Health Care Needs of the Deaf Community Through Medical Interpreters,” American Family Physician, Oct 2024.

[2] Mitchell & Young, “How Many People Use Sign Language?” NHIS 2010–2018; RIT Libraries summary.

[3] Registry of Interpreters for the Deaf, ~10,000 certified interpreters, cited by Deaf Services Unlimited.

[4] US Dept. of Justice, “ADA Business Brief: Communicating with People Who Are Deaf or Hard of Hearing in Hospital Settings.”

[5] National Association of the Deaf, “Position Statement on Health Care Access for Deaf Patients.”

[6] Hazelwood et al., Journal of Clinical Nursing, Oct 2026.

[7] Wilson-Menzfeld et al., Health & Social Care in the Community, 2025.

[8] Pennsylvania Patient Safety Authority, May 2025.

[9] Gallaudet University, “How Many People Use ASL in the United States?”

[10] Akeely et al., Cureus, Nov 2022. n = 166 emergency physicians.

[11] Reeves et al. (2002), cited in Frontiers in Communication, 2020.

APPENDIX A2

The competitive landscape, named.

PLAYER	WHAT THEY DO	WHY THEY ARE NOT THIS
VRI providers	Video remote interpreting, human interpreters	Human, scheduled, per-minute. We cover the encounters that never get booked.
Interpreter agencies	Supply qualified humans	Referral channel, not a competitor. They hold the hospital relationships.
Google SL2T	Best-in-class ASL→English, Pixel 11 only	No API, no SDK, and medical use explicitly excluded from its intended use.
Signapse	Photorealistic English→ASL generation	Generation only. Their own docs confirm no recognition direction.
Sign-Speak / SignCaption	Two-way recognition and avatars	General purpose. Consumer terms are non-commercial and train on uploaded content.
Unmute	Clinical ASL, bounded vocabulary, measured	Narrow on purpose, so an accuracy number is possible at all.

APPENDIX A3

Regulatory posture.

The ADA requires hospitals to provide effective communication, including a qualified interpreter where necessary for meaningful access. Unmute sits alongside that obligation, addressing the routine encounters where no interpreter was ever booked.

Unmute does not satisfy the requirement for a qualified interpreter and is not positioned as doing so.

Consent for procedures, diagnosis disclosure, mental health assessment and end-of-life discussions remain human-interpreter conversations. This positioning must be reviewed by US healthcare counsel before external use.

APPENDIX A4

Data ethics and consent.

HOW THE CORPUS IS COLLECTED

- Consented, paid Deaf participants and qualified interpreters
- Recorded for vocabulary, not for patient encounters
- Annotated to a protocol owned by an ASL linguistics advisor
- Regional variation preserved, not averaged away

WHAT WE WILL NOT DO

- Train on patient data from pilots
- Accept terms that grant a vendor rights to our recordings
- Scrape existing sign-language dictionaries
- Publish a model we cannot explain the data provenance of

APPENDIX A5

What we are deliberately not doing.

× **Multilingual**
ASL first, because the data and the benchmark exist, and because the buyer with budget exists.

× **Avatar synthesis**
A generative avatar producing signed English is a liability in a clinical setting.

× **EHR integration**
Valuable, not load-bearing for a first pilot, and a long procurement cycle.

× **Consumer app**
The buyer is a hospital. A consumer product would change the entire compliance story.

× **A general translator**
Unbounded vocabulary makes an accuracy number impossible to state.

× **Replacing interpreters**
Never. The product covers what nothing currently covers.

APPENDIX A6

Why not Nigeria first?

ASL was chosen because the published clinical evidence, the open datasets, and the payer with budget all exist there. Nigerian Sign Language is the founder's home context and a genuine second-market ambition.

NSL is not a current capability.

ASL and NSL are distinct languages with different grammar and vocabularies. Nothing in this deck, and nothing in the v1 product, applies to Nigerian Sign Language without a separate corpus and validation.